

# SUMMARY OF PRODUCT CHARACTERISTICS (SmPC) — EU English (Synthetic / Mock)

**Product:** Cardiomet 20 mg / 40 mg film-coated tablets (nefrosartan)

**Document ID:** SmPC-EU-EN-CARDIOMET-NEFROSARTAN-v0.2 (Synthetic)

**Date of revision (synthetic):** 16 Jan 2026

Synthetic mock SmPC aligned to EMA QRD v10.4 heading structure for prototyping only.

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## 1. NAME OF THE MEDICINAL PRODUCT

Cardiomet 20 mg film-coated tablets

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## 2. QUALITATIVE AND QUANTITATIVE COMPOSITION

Each film-coated tablet contains nefrosartan (as nefrosartan potassium) 20 mg or 40 mg.

Excipient with known effect: lactose monohydrate.

For the full list of excipients, see section 6.1.

## 3. PHARMACEUTICAL FORM

Film-coated tablet.

Cardiomet 20 mg: oval, white, debossed “NM20” (synthetic).

Cardiomet 40 mg: oval, light blue, debossed “NM40” (synthetic).

## 4. CLINICAL PARTICULARS

### 4.1 Therapeutic indications

Cardiomet is indicated for the treatment of essential hypertension in adults.

### 4.2 Posology and method of administration

#### Posology

Adults: The recommended starting dose is 40 mg once daily. The usual maintenance dose is 40–80 mg once daily. The maximum dose is 160 mg/day.

Elderly: No routine dose adjustment; consider a lower starting dose in frail patients or those at risk of hypotension.

Renal impairment: Mild/moderate—no adjustment generally required. Severe impairment (eGFR <30 mL/min/1.73m<sup>2</sup>) or dialysis—use with caution and

monitor renal function and potassium.

Hepatic impairment: Mild—caution. Moderate—start low and titrate cautiously. Severe hepatic impairment—contraindicated.

#### **Method of administration**

Oral use. Swallow with water, with or without food.

### **4.3 Contraindications**

- Hypersensitivity to the active substance or to any of the excipients listed in section 6.1
- Pregnancy in the second and third trimester (see section 4.6)
- Severe hepatic impairment / cholestasis
- Concomitant use with aliskiren-containing products in patients with diabetes mellitus or renal impairment (eGFR <60 mL/min/1.73 m<sup>2</sup>)

### **4.4 Special warnings and precautions for use**

Hypotension/volume depletion; renal function impairment; hyperkalaemia; dual RAAS blockade; angioedema; aortic/mitral stenosis/obstructive cardiomyopathy; primary aldosteronism; hepatic impairment.

This medicinal product contains lactose. Patients with rare hereditary problems of galactose intolerance, total lactase deficiency or glucose-galactose malabsorption should not take this medicinal product.

### **4.5 Interaction with other medicinal products and other forms of interaction**

Potassium-sparing diuretics/potassium supplements/salt substitutes: risk of hyperkalaemia.

NSAIDs: reduced antihypertensive effect; risk of renal impairment—monitor renal function.

Lithium: increased lithium levels/toxicity possible—avoid or monitor lithium levels.

Other antihypertensives: additive blood pressure lowering.

### **4.6 Fertility, pregnancy and lactation**

#### **Pregnancy**

Use of angiotensin II receptor antagonists is not recommended during the first trimester of pregnancy. Use is contraindicated during the second and third trimesters of pregnancy. If pregnancy is detected, Cardiomel should be discontinued and alternative treatment started.

**Breast-feeding**

Use is not recommended. Alternative treatments with better established safety during breast-feeding are preferable, especially while nursing a newborn or preterm infant.

**Fertility**

No clinically meaningful effects anticipated (synthetic).

**4.7 Effects on ability to drive and use machines**

Cardiomel may cause dizziness or fatigue, particularly at initiation or during dose increases. Patients should be cautioned about driving or using machines if affected.

**4.8 Undesirable effects**

Most commonly reported: dizziness, headache, fatigue. Clinically relevant: hypotension, hyperkalaemia, increased creatinine; rare angioedema.

**Tabulated list of adverse reactions (synthetic)**

| System<br>Organ<br>Class                             | Very<br>common | Common                  | Uncommon                        | Rare                      | Not<br>known |
|------------------------------------------------------|----------------|-------------------------|---------------------------------|---------------------------|--------------|
| Nervous<br>system<br>disorders                       |                | Dizziness;<br>Headache  |                                 |                           |              |
| Cardiac<br>disorders                                 |                |                         | Palpitations                    |                           |              |
| Vascular<br>disorders                                |                | Hypotension             | Orthostatic<br>hypoten-<br>sion | Syncope                   |              |
| Gastrointestinal<br>disorders                        |                | Nausea;<br>Diarrhoea    | Dyspepsia                       |                           |              |
| Renal and<br>urinary<br>disorders                    |                | Creatinine<br>increased |                                 | Acute<br>kidney<br>injury |              |
| Metabolism<br>and<br>nutrition<br>disorders          |                | Hyperkalaemia           |                                 |                           |              |
| Skin and<br>subcuta-<br>neous<br>tissue<br>disorders |                |                         | Rash;<br>Pruritus               | Angioedema                |              |

### **Reporting of suspected adverse reactions**

Reporting suspected adverse reactions after authorisation of the medicinal product is important. It allows continued monitoring of the benefit/risk balance. Healthcare professionals are asked to report any suspected adverse reactions via the national reporting system listed in Appendix V.

### **4.9 Overdose**

Supportive treatment; monitor blood pressure, electrolytes, renal function. Dialysis unlikely to be effective (synthetic).

## **5. PHARMACOLOGICAL PROPERTIES**

### **5.1 Pharmacodynamic properties**

Pharmacotherapeutic group: angiotensin II receptor antagonists, plain (synthetic).

ATC code: not assigned (synthetic).

Mechanism: selective AT1 antagonism → reduced vasoconstriction and aldosterone → reduced blood pressure.

### **5.2 Pharmacokinetic properties**

Absorption Tmax 1.5–3 h; bioavailability ~35%; protein binding ~98%; terminal half-life ~12 h; elimination mainly biliary/fecal with minor renal component (synthetic).

### **5.3 Preclinical safety data**

Embryo-fetal toxicity consistent with RAAS inhibition; no genotoxic signal in standard assays (synthetic).

## **6. PHARMACEUTICAL PARTICULARS**

### **6.1 List of excipients**

Tablet core: lactose monohydrate, microcrystalline cellulose, croscarmellose sodium, magnesium stearate.

Film coat: hypromellose, macrogol, titanium dioxide, iron oxides (synthetic).

### **6.2 Incompatibilities**

Not applicable.

### **6.3 Shelf life**

3 years.

#### **6.4 Special precautions for storage**

Store below 30°C. Keep in the original package to protect from moisture.

#### **6.5 Nature and contents of container**

PVC/Alu blister packs: 28, 30, 56, 90 tablets (not all pack sizes may be marketed) (synthetic).

HDPE bottle with desiccant: 30, 90 tablets (synthetic).

#### **6.6 Special precautions for disposal and other handling**

No special requirements.

### **7. MARKETING AUTHORISATION HOLDER**

Acme Pharma Europe B.V., Example Street 1, 1234 AB, Amsterdam, Netherlands (synthetic).

### **8. MARKETING AUTHORISATION NUMBER(S)**

EU/1/26/000/001 (20 mg)

EU/1/26/000/002 (40 mg) (synthetic).

### **9. DATE OF FIRST AUTHORISATION/RENEWAL OF THE AUTHORISATION**

16/01/2026 (synthetic).

### **10. DATE OF REVISION OF THE TEXT**

16/01/2026 (synthetic).